

- If the obstruction has not cleared and he becomes unconscious, call for emergency help, then start CPR with chest compressions. Continue until help arrives.

FIT AND/OR EPILEPTIC SEIZURE (CONVULSIONS)

Convulsions are seizures or fits that cause loss of consciousness and a jerking movement of muscles. They are caused by changes in the normal nerve impulses to and from the brain. High fever, certain medications, injury to the head, infection of spinal fluid or fluid surrounding the brain, or a tumor in the spine or brain all could result in seizures.

What you need to know

Convulsions can be frightening, especially if you have not seen them before. They will stop on their own after a few seconds or minutes.

- The person cries or moans, then loses consciousness.
- The eyes stare blankly or roll back.
- There may be sudden loss of control of bladder and bowels.
- Jerking movements of the body, especially the arms and legs, occurs.
- The seizure may involve the whole body or just one part.
- If there is a possibility of a convulsion, the doctor may order a medication to help prevent it.

First Aid: Convulsions



How you can offer care

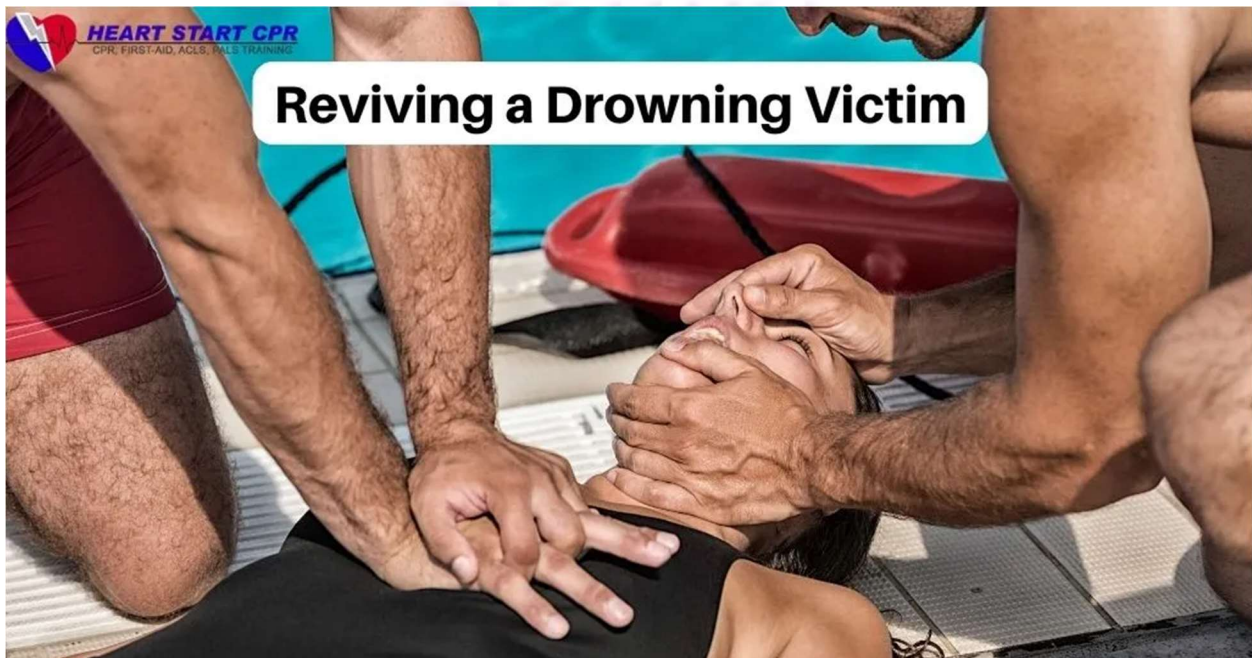
Remain calm. Once a seizure has started you cannot make it stop.

- Prevent a fall to the floor by cradling the person in your arms if the seizure occurs while the person is in bed or a chair.
- Move objects that might injure the person.
- Remove eyeglasses and loosen tight clothing.
- Do not try to restrain movement. Let the seizure happen.
- Try to turn the head to the side so saliva can flow out of the mouth, not down the throat.
- Do not try to force open the jaw or put a hard object into the mouth to keep it open. This is dangerous. You may damage the teeth or gums. Also the person may break whatever you use in the mouth and may choke.
- Try to notice what type of movement the person makes, how long the seizure lasts and which parts of the body move with the seizure.
- Stay with the person until the seizure stops. When it does, make sure the breathing is all right.
- Speak gently and reassuringly as the person may be confused or frightened and likely will not remember what happened.

- Be aware that convulsions are exhausting. The person may have a headache and will be very tired and need rest.
- Use side rails and bumper pads on the bed if someone may have seizures. Stay close when the person is walking or sitting in a chair.

DROWNING

Drowning can result in death from *hypothermia* due to immersion in cold water, sudden cardiac arrest due to cold water, spasm of the throat blocking the airway and/or inhalation of water and consequent airway obstruction. A casualty rescued from a drowning incident should always receive medical attention even if he seems to have recovered at the time. Any water entering the lungs causes them to become irritated, and the air passages may begin to swell several hours later—a condition known as secondary drowning. The casualty may also need to be treated for hypothermia.



Your Aim;

- To restore adequate breathing.
 - To keep the casualty warm.
 - To arrange urgent removal to the hospital.
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1. If you have rescued the casualty from the water, help him lie down on a rug or coat with his head lower than the rest of the body so that water can drain from his mouth. This reduces the risk of inhaling water.
 2. Treat the casualty for hypothermia; replace wet clothing with dry clothes if possible and cover him with dry blankets or coats. If the casualty is fully conscious, give him a warm drink.
 3. Call for emergency help even if he appears to recover fully because of the risk of secondary drowning. Monitor and record his vital signs—level of response, breathing, and pulse until help arrives.
 4. If the casualty is unconscious and you are on your own, give CPR for two minutes before you call for emergency help.
 5. If an unconscious casualty starts to breathe, treat him for hypothermia by covering him with warm clothes and blankets. If he recovers, replace his wet clothes with dry ones if possible and cover him with warm clothes and blankets. Monitor and record vital signs—breathing, pulse and level of response until help arrives.